

**STRATEGY & OPERATIONS**

Maternalink Pricing: Per Product And Annual Service

Vytalix company-build documentation · reference
21_MATERNALINK_PRICING

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

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Contents

Executive view	1
1. Product catalogue (what is sold)	2
2. Cost-to-serve (why the prices are what they are)	3
3. Price book A — United Kingdom (ESTIMATE)	4
4. Price book B — Africa and programme funders (ESTIMATE)	5
5. Commercial rules	6
6. The Maternity Watch: build, buy or partner	7
7. Validation plan (before any price is quoted externally)	8
Priorities / Risks / Next actions	9

Owner: CFO with Chief Product Officer · **Status:** PROPOSED v1.0 (8 September 2026) · **Every figure is an ESTIMATE until validated with three buyers per segment** · Supersedes the £300/woman/year figure in the March 2026 Ekiti proposal, which is retired. Governing files: [/FACTS_BASE.md](#) , [/docs/01_BUSINESS_MODEL.md](#) , [/docs/08_FINANCIAL_MODEL.md](#) .

Executive view

1. MaternaLink is priced as **a platform licence plus optional modules, plus an annual service plan**, with hardware (the Maternity Watch) priced separately as a per-device lease or purchase. This separates software value from device cost and lets buyers start without hardware.
2. Two price books: **UK** (NHS trusts and ICBs, private maternity, employers) and **Africa / programme** (state ministries, NGOs, insurers), because willingness to pay and cost-to-serve differ by an order of magnitude.
3. Indicative annual software price per woman enrolled: **UK £18–£36; Africa programme £4–£10** at scale, with a pilot minimum so no contract is below cost. The old £300/woman/year figure is 8–30× these levels and is not defensible against comparable programmes.
4. Annual service is priced at **18–22% of licence value** (Standard) rising to 30–35% (Premium, 24/7 clinical-safety on-call and named account management), in line with enterprise health-software norms (ESTIMATE).
5. Gross margin target: software 75–85%; services 45–55%; hardware 20–30% (hardware is a channel, not a profit centre).

1. Product catalogue (what is sold)

Code	Product	What the buyer gets	Unit	Regulatory posture
ML-CORE	MaternaLink Core platform	Patient registry (mother + newborn), offline-first capture of symptoms and vitals, timeline, clinician notes, escalation queue, supervisor dashboards, EN/FR interface, exports, audit log	Per site licence + per enrolled woman	Non-diagnostic care-coordination software; intended-use statement controls scope
ML-TRIAGE	Maternity Triage module	Structured triage intake for maternity assessment units, prioritisation rules configured by the site's clinical governance, contact and outcome logging, waiting-time analytics	Per site	Rules configured and owned by the provider; Vytalix supplies the rule editor, not the rules. Classification TO VALIDATE with MHRA guidance before any rule ships pre-configured
ML-ENGAGE	Woman & partner engagement module	Daily questionnaire, symptom log (including itching of hands and feet, vomiting, headache, visual disturbance, reduced movements), education content signed off by clinicians, partner quizzes and prompts, appointment reminders via SMS/WhatsApp	Per enrolled woman	Non-diagnostic; symptom logs route to a human. No automated diagnosis
ML-GLUCOSE	Diabetes-in-pregnancy tracking	Self-reported blood-glucose entries with target ranges set by the clinician, trend view, threshold notifications to the care team	Per enrolled woman with the condition	Threshold notifications configured by the clinician; classification TO VALIDATE (likely needs a device pathway if the software interprets readings)

Code	Product	What the buyer gets	Unit	Regulatory posture
ML-VOICE	Voice diaries	Recorded diaries and voice notes attached to the timeline; transcription and translation as a roadmap option	Per enrolled woman	Non-diagnostic storage and playback only. "Voice tracing of baby movements" is a research track , not a sold feature, until validated
ML-WATCH	Maternity Watch (device)	A wearable issued at the 16-week appointment and returned at postnatal discharge; captures pulse, activity and prompts the daily check-in; syncs to the platform	Per device (lease or purchase) plus per-device service	A wearable that measures physiological parameters for clinical use is very likely a medical device ; Vytalix should procure a UKCA/CE-marked device from an existing manufacturer and integrate it, rather than manufacture (see \$6)
ML-INTEGRATE	Integration pack	Interfaces to the provider's maternity record (e.g., BadgerNet, K2, Euroking), DHIS2 alignment for programmes, HL7/FHIR endpoints, single sign-on	Per interface, one-off plus annual	Standard software integration
ML-ANALYTICS	Programme analytics	Population dashboards, defaulter tracing, facility performance, exportable indicators	Per programme	Non-clinical analytics
ML-TRAIN	Training and e-learning	Role-based training for midwives, community health workers, operators; refresher modules; certification of completion (not accredited)	Per cohort or per learner	E-Learning pillar

Replacement versus integration: where a hospital has no digital maternity record, MaternaLink Core can act as the working record for the programme (replace); where one exists, MaternaLink integrates and never becomes the legal record (integrate). Price the integration pack accordingly.

2. Cost-to-serve (why the prices are what they are)

Cost line	UK per woman per year	Africa programme per woman per year	Notes
Hosting, storage, backups	£0.60–£1.20	£0.30–£0.60	UK region; Nigeria residency adds 20–40%
Messaging (SMS/WhatsApp)	£1.50–£3.00	£0.80–£2.00	~40 messages per pregnancy; Nigeria SMS via aggregator
Support and clinical-safety overhead (allocated)	£3.00–£5.00	£0.60–£1.20	Includes hazard-log maintenance, incident handling
Onboarding and training (amortised)	£2.00–£4.00	£0.50–£1.00	Higher touch in UK trusts
Payment/collection, compliance	£0.40	£0.20	
Total cost-to-serve	£7.50–£13.60	£2.40–£5.00	Floor prices below
Floor price (no contract below)	£14	£4	Pilot minimums in \$3 protect the floor

Watch hardware (if used): device £60–£140 unit cost for a suitable wearable from an established manufacturer (ESTIMATE), 24–30 month life, 10–15% loss/damage, refurbishment £8–£15 per cycle. Cost per pregnancy served

≈ £35–£70. Priced at lease £4–£7 per woman per month or purchase with service plan.

3. Price book A — United Kingdom (ESTIMATE)

Per product

Product	Pricing model	Indicative price	Notes
ML-CORE	Site licence + per enrolled woman per year	Site licence £12,000–£30,000/year (by births: <3,000 / 3,000–6,000 / >6,000) plus £18–£36 per woman per year	A 5,000-birth trust at £22/woman ≈ £110k + £20k licence = £130k/year
ML-TRIAGE	Per site per year	£15,000–£35,000	Includes rule editor, two configuration workshops
ML-ENGAGE	Per enrolled woman per year	£8–£14	Bundled discount 20% with Core
ML-GLUCOSE	Per enrolled woman with condition per year	£20–£30	Typically 5–10% of the cohort
ML-VOICE	Per enrolled woman per year	£3–£5	Storage included up to 60 minutes per pregnancy
ML-WATCH	Lease per device per month; or purchase	£4–£7 per month per device (24-month term) or £120–£220 purchase + £30/year service	Includes replacement pool 10%
ML-INTEGRATE	One-off per interface + annual	£15,000–£40,000 setup; £4,000–£8,000/year	Maternity record vendors may charge their own fees (TO VALIDATE)
ML-ANALYTICS	Per site per year	£6,000–£12,000	Included in Enterprise bundle
ML-TRAIN	Per cohort (up to 25)	£2,500–£4,500	E-learning licence £40–£90 per learner per year
Implementation	One-off	£25,000–£75,000 per site	Clinical-safety case, DPIA support, configuration, go-live
Pilot (12 weeks, one site, up to 300 women)	Fixed	£40,000–£90,000	Includes evaluation report; converts to annual on success

Annual service plans (UK)

Plan	Price (% of annual licence)	Includes
Standard	18–22% (minimum £8,000)	Business-hours support, quarterly releases, security patching, hazard-log maintenance, uptime target 99.5% (design target, not a guarantee until measured)
Enhanced	25–28%	Standard plus extended hours 07:00–22:00, named account manager, two configuration changes per quarter, quarterly service review, DSPT evidence pack
Premium	30–35%	Enhanced plus 24/7 severity-1 response, clinical-safety officer on-call, annual clinical-safety case refresh, dedicated environment option, uptime target 99.9%

Enterprise bundles

Bundle	Contents	Indicative annual (5,000-birth trust)
Coordinate	Core + Engage + Standard service	£150k–£190k
Coordinate + Triage	Above + Triage + Analytics + Enhanced service	£210k–£270k
Full programme	Above + Glucose + Voice + Watch lease for 20% of cohort + Premium service	£330k–£420k

4. Price book B — Africa and programme funders (ESTIMATE)

Product	Pricing model	Indicative price	Notes
ML-CORE	Per enrolled woman per year, tiered by volume	£10 (up to 5,000) · £7 (5,001–25,000) · £5 (25,001–100,000) · £4 (100,000+)	Includes programme dashboard; site licence waived for public programmes
ML-ENGAGE (SMS/WhatsApp)	Per enrolled woman per year	£3–£5	Message costs passed through at cost above 40 messages
ML-TRIAGE	Per facility per year	£1,500–£4,000	Facility-level rules owned by the programme's clinical lead
ML-GLUCOSE	Per enrolled woman with condition	£6–£10	
ML-VOICE	Per enrolled woman	£1–£2	Local-language transcription roadmap
ML-WATCH	Per device	Lease £2–£4/month or purchase £90–£160 + £15/year service	Only where a programme funds hardware; not in base offers
ML-INTEGRATE (DHIS2 / HMIS)	One-off + annual	£8,000–£25,000 + £3,000/year	
ML-TRAIN	Per cohort of 30	£1,200–£2,500	Train-the-trainer model
Implementation	One-off per state or programme	£40,000–£120,000	Includes data-protection registration support, local hosting decision, baseline evaluation design
Annual service	20% Standard · 28% Enhanced · 35% Premium of software value	Minimum £12,000	In-country support partner required above 25,000 women (partner margin included)

Worked example: a 15,000-woman state programme (year 1)

Line	Amount
Core 15,000 × £7	£105,000
Engage 15,000 × £4	£60,000
Triage 20 facilities × £2,500	£50,000
Implementation (one-off)	£80,000
Training 10 cohorts	£18,000
Enhanced service (28% of £215k software)	£60,200

Line	Amount
Year-1 total	£373,200 (≈ £25 per woman)
Year-2 run-rate	≈ £275,000 (≈ £18 per woman)

Compared with £4.5m for the same cohort in the retired proposal. Gross margin at these prices ≈ 62% year 1, 74% year 2 (ESTIMATE).

Worked example: a 5,000-birth UK trust, Coordinate + Triage bundle

Line	Amount
Core licence £20,000 + 5,000 × £22	£130,000
Engage 5,000 × £10	£50,000
Triage	£25,000
Analytics	£8,000
Enhanced service (26% of £213k)	£55,400
Implementation (one-off, year 1)	£45,000
Year-1 total	£313,400 · year-2 run-rate £268,400

5. Commercial rules

- Pilot first, always: a fixed-price pilot with an evaluation report, converting to an annual contract with pilot fees credited 50% against year one.
- Minimum contract value: £40,000 (UK), £25,000 (programme), to protect the floor and service quality.
- Multi-year: 5% discount for 3-year terms; annual uplift capped at CPI + 2%.
- Volume discounts only via the published tiers; no ad hoc discounting below floor.
- Payment: UK annual in advance (public sector on receipt of PO); programmes 40% on signature, 40% on go-live, 20% at 6 months, or donor schedule.
- Hardware: never bundle device cost into the per-woman software price; show it as its own line so software margin is visible to investors and price comparisons to competitors are fair.

6. The Maternity Watch: build, buy or partner

Option	Pros	Cons	Verdict
Manufacture a Vytalix device	Brand, margin	Medical-device manufacturer obligations (UKCA, QMS ISO 13485, post-market surveillance), £1m+ and 24+ months	KILL for now
Integrate an existing UKCA/CE-marked wearable (e.g., a clinically certified wristband) under a supply agreement	Fast, compliant, priced as a pass-through	Lower margin; dependency	BUILD (partner) once v1 software is live
Consumer wearable (non-medical) for prompts and activity only	Cheap	Cannot be used for clinical vitals; expectation mismatch	TEST for engagement-only use (reminders, daily check-in)

7. Validation plan (before any price is quoted externally)

1. Three UK maternity digital leads: reaction to £18–£36 per woman and to bundles (Day 31–60 discovery interviews).
2. Two Nigerian programme leads and one NGO: reaction to £4–£10 per woman and the £25-per-woman year-1 all-in.
3. One insurer/HMO: appetite for per-member pricing.
4. Cost-to-serve check after the prototype audit (hosting, messaging actuals).
5. Regulatory opinion on Triage, Glucose and Watch modules before they are priced in any proposal.

Priorities / Risks / Next actions

Priorities: Core + Engage pricing validated first; Triage and Glucose only after the regulatory opinion; Watch as a partner device.

Risks: buyers anchor on free donor-funded programmes (mitigation: pilot-to-contract with evidence; complementarity positioning); under-pricing service (mitigation: minimums); device classification creep.

Next actions: put this price book into `/assets/PRICING_STRUCTURE.md` as the MaternaLink section; add price fields to the proposal template; test the two worked examples in the Day 31–60 interviews.